

Reflective Paper

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I have no known conflict of interest to disclose.

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Abstract

Counseling is a dynamic field. No counselor can expect to graduate, pass licensure, and never look back. The field is replete with accountability for ethical decisions, treatment, and client care. Even with all this educational and continuing educational emphasis on evidence-based, intentional, individual counseling, counselors grow complacent; I am no exception. This paper is a discussion of the ways I currently view and ways I have to grow in theories, assessment, diagnosis, case conceptualization, evidence-based practice, effective treatment, and ethical competencies. My hypothesis is that I am both a good counselor and have many areas and ways to grow in the future. The findings hold true to that hypothesis.

Keywords: Ray Wagoner, counseling, theories, areas of growth

Reflective Paper

Writing this paper and all that entails has been a breath of fresh air for me. For the past year as I have been a full-time counselor, I have had my head down and not paid much attention to the rest of the counseling world. I can give myself grace since I have been overwhelmed with clients and being a church staff member. I can give myself time to adjust. But I must lift my head up and look around. Writing this paper has helped me address and readdress why I do what I do. What follows is a much-needed personal reflection on the aspects of counseling that I need to grow in.

Theories

My counseling approach stems from an integration of two separate but clarifying lenses. The first lens I use is philosophical in nature. It is a worldview based on my Christian religion and relationship with God. With this philosophy I want to point people to a higher power outside of themselves and outside of me as their guide. I want clients to draw upon the same hope and security for life and living that I do. The second lens I use is counseling psychology. Blending these two lenses is not impossible nor improbable. In fact, I find counseling psychology has a great deal in common with Christian Scriptures' teaching. The most profound similarity I have found is with Bowlby's attachment theory. While Interpersonal Psychotherapy, as seen through the lens of Bowlby's attachment theory, can be a powerful relational tool in repairing mental illness (Ravitz & Watson, 2014), I find the idea that human beings need a secure base is quite true to Scriptures such as Ps 34:18 and Heb 13:5-6 (English Standard Version, 2001) as well. Each of these passages teach that God the Father invites us into a relationship with Him and is constantly near us as we meet life's difficulties. Attachment theory teaches that security comes as we are assured that others will come alongside us as we face trauma and pain. Both sources call for "stronger," "wiser,"

“other” (R. Rana, 2019) to return when new, potentially traumatizing, circumstances arise. A meta-research analysis conducted by Rowe et al. (2020) showed that priming clients to attachment figures created secure attachments where none existed before in an overwhelming number of studies. What this study showed is that we need to help clients find that secure base in session so they can find it outside of session. What this study didn’t account for was that the most important secure base of all is found in God the Trinity.

When searching for a theory to ground myself in, attachment theory found me. Working through my own emotional and mental health, I found that I was soothed from anxiety, healed from trauma, and purged of self-defeating thoughts when others would validate and normalize my struggle. I soon realized what Schore and Schore discovered in their 2008 research project; attachment is a key part of understanding the development of human emotional growth and is therefore pertinent to the current modern treatments of emotional and mental health. I use attachment theory and the interventions related to Bowlby’s understandings and to interpersonal interventions because they are evidence-based, effective, true to my biblical worldview, and come naturally to my way of helping others.

Preparing for this paper has reminded me that my approach to counseling is valid and helpful. Preparing for this paper has also reminded me that I have a lot to learn. The reason I am in the Ph.D. program at Liberty is to learn what I need to learn so I can help others learn as well. This brings me to the skills I am most lacking in, assessment, diagnosis, and treatment planning.

Phases of Counseling

Comprehensive Assessment

I believe assessment is one of the skills I need the most improvement in. This is likely true simply because I have not encountered the type of clientele that would force me to use

assessments as much in my professional counseling setting. As a staff counselor on a local church staff, the clients I typically see are not diagnosed, not escalated, and not looking for typical therapy. In addition to clientele, I honestly do not have the accountability to assess, diagnose, and follow strict treatment plans from supervisors and insurance networks. Although I have not seen this as a deficit to effective counseling in the past, reading for this class, researching for, and conceptualizing this paper have prompted me to reengage these aspects of counseling. On the other hand, as I write this paper, I do find myself fighting old enemies of fear of rejection and imposter syndrome. It is difficult to convince myself that education and experience would fill in the gaps I have with assessment. Allowing myself ambivalence over being the perfect counselor and the perfect student has given me the freedom to ask myself what I would like to learn and use for assessment, diagnosis, and treatment planning in the future.

One component of assessment that I do well is focusing on a healing relationship with clients. Part of my hesitation to use formalized assessment is the fear of feeling too mechanical, clinical, and processed. I want my clients to know they are meeting with a human being. I find that a good interview process that includes the Biopsychosocial model is a good way to help alleviate these concerns. Asking questions about sleeping patterns, physical/medical concerns and recent doctor's visits, and support system reliability is a good way to evaluate hidden mental health concerns. Combining this discrete interview with my intake and informed consent allows me to paint a picture for clients of what can be expected in counseling. When necessary, I can refer clients to Psychiatrists, Family Medical Practitioners, or colleagues who specialize in areas of mental health I am not trained or skilled in. Continuing to follow this practice and including an element of reassessment in each session will help me stay on track with my treatment plan. I also need to be proactive in discovering formal assessments I can use intentionally.

Many if not all of my clients present with both anxiety and depression. Some have other complaints such as addiction, affair betrayal, marriage distress, and grief and need treatment for these as well. As I consider the assessment phase of treatment, I most generally find concern with anxiety and depression, however. For the assessment of anxiety, I understand that the General Anxiety Disorder 7 (GAD-7) is reliable, valid, and takes little time to complete. This self-report screen is widely used and appropriate for many different populations (Anthony & Barlow, 2020). A short, repeatable instrument like the GAD-7 would make intakes more informative and help me conceptualize clients. As a widely used and understood instrument the, GAD-7 would be useful in collaborating with colleagues and other medical professionals. I intend to find ways to better understand the GAD-7 and its implications for treatment throughout my education. I believe it could be an important part of client care going forward.

The other presenting problem I am most familiar with is depression. I generally find people anxious about their depression. I believe, anecdotally, that some unhealthy faith messages still haunt people around depression. Many clients have a fear that being diagnosed with depression will mean they are not spiritual enough or not a good enough Christian. In order to continue my relational style and ease any concerns around the word depression, I first want to assess Biopsychosocial aspects similarly to anxiety. I want to know about sleeping patterns, eating patterns, physical conditions, and support system/isolation. If there are any new or worsening changes to these patterns or any new illnesses associated with the onset of the depressive state, I want the client to see a primary care physician to make sure there are no hidden underlying conditions. This assessment is also helpful when the client's depression seems mild and appropriate to a new or changing circumstance.

Digging deeper into an assessment specifically for depression and looking for something that is repeatable, concise, and transferable, I found the Beck Depression Inventory (BDI). Beck et al. (1988) reviewed 26 years of studies concerning the BDI and found it to be valid, reliable, and typically generalizable to diverse populations. I currently know little about this instrument and its implications for treatment. I am interested in finding ways to increase my knowledge and utilize the BDI throughout the phases of counseling.

Determining an accurate DSM-5 diagnosis

As I have mentioned before, my current setting does not require or encourage the use of diagnosis in intake or in any of the phases of treatment. I understand this rationale to be client-centered and outpatient clinic preferred. I agree with the philosophy that giving a client a label is not always helpful. I do, however, have clients who see improvement in symptoms through their understanding of diagnosis. It appears their symptoms make more sense when connected to a verifiable outcome. Currently, the only help I have for them to find these outcomes is to read through and familiarize myself with the Diagnostic and Statistical Manual of Mental Disorders and make the best connection to symptoms that I can. When looking for a better instrument for clinical diagnosis, I came up short. According to Daily et al. (2014), the American Psychological Association (APA) has not published specific assessments for diagnosing to DSM criteria. The rationale for this seems to be that the printing of assessments does not allow for correction and updating as often as they saw might be necessary. Daily et al. also mention that the best tools for diagnoses are found on the APA website. I would concur as I found searches on assessments for specific diagnosis through the APA's website profitable. One does have to be aware that there is no instrument that will take a clinician from no diagnosis to a specific diagnosis in and of itself. One must be able to closely identify possible diagnoses and follow APA guidelines and

recommendations for assessing for each of these illnesses. That being said, I would have a lot of learning to do if my practice setting changed tomorrow. Thankfully, I don't see that happening anytime soon.

Structure and Phases of Treatment

As a part of moving from phase to phase of treatment, I would like to be more qualified in the Outcome Rating Scale (ORS) developed by Scott Miller. According to a study by Miller et al. (2003), the ORS is consistent and accurate enough to be trusted in assessing change in a client after interventions have been utilized. It is true that all brief measures lack the consistency of more in-depth measures, however, this is another instrument that fits with my relational style and allows me to reassess consistently without becoming overly clinical or clunky.

According to yourceus.com (2022) and a class titled *Components of effective counseling*, "The basic stages of counseling are: 1) Developing the client/clinician relationship; 2) Clarifying and assessing the presenting problem or situation; 3) Identifying and setting counseling or treatment goals; 4) Designing and implementing interventions; and 5) Planning, termination, and follow-up". As I read this list, I not only agree but find that I follow these phases/stages of treatment already.

Case Conceptualization

The list above is not only my idea of phases of treatment but is closely tied to conceptualizing each case. The difference between the two concepts for me would be the difference between a broad overview and a detailed plan. Conceptualizing a client would be the broad overview of how to proceed. The concept of treatment would even include the phases of treatment in this definition. When beginning a case conceptualization, I draw from the article by Prieto and Scheel (2002) that describes using Signs and Symptoms, Topics of Discussion,

Interventions, and Progress and Plan (STIPS) to better understand what each client needs and how to treat, evaluate and reevaluate through the treatment process. Using the STIPS method keeps me on track and accountable to a plan. I must admit that I have more urgency at the outset of treatment than in the ongoing intervention stage. Planning and sticking to it is natural for me. It is the question below of evaluating that I have difficulty with.

Evidence Based Practices

Counselors need to stay up to date with the latest in evidence-based treatment to be the most effective in treating clients as Paradise and Kirby suggest in their summation of Hollon et al. (2002) from 2011. The rationale provided by Paradise and Kirby is that no one client is the same. For a counselor to be effective across cultures, genders, diagnoses, presenting issues, client goals, and the like, she must be able to track and self-educate on the most specific research applicable. Similarly, new research and interventions are emerging daily. If one finds it helpful to stay up to date with the latest trends in marketing for his practice, one will certainly find it wise and ethical to stay abreast of best practices in treating clients. The examples below are cases where a one-size-fits-all approach is both unethical and unhelpful.

A Nine-Year-Old Suffering with OCD

Take for example a nine-year-old suffering from obsessive-compulsive disorder (OCD). This case concept is a purposefully specific case. Treating early onset OCD, the same as adolescent OCD or adult OCD may seem straightforward, but it is far from it. One cannot simply assume that interventions and outcomes are generalizable across ages. I was prepared to find research that identified specific interventions for childhood OCD. However, a very interesting study by Nakatani, et al. (2001) revealed that early onset OCD should be treated the same as adult OCD.

The next obvious specific identifier, in this case, would be the diagnosis. Again, treating OCD with the same interventions as anxiety or attachment disorder may seem valid, however, this assumption is not one I'm willing to stake a client's care on. In researching evidence-based interventions for early-onset OCD I found that Geller and March recommend that cognitive behavioral therapy (CBT) (including elements of exposure and response prevention) be used as the most reliable and valid treatment for childhood and adolescent OCD. Once again, I was stumped to find that previously adult-centered theories and treatments were generalizable to a nine-year-old with the same diagnosis.

The question remains, what does an interpersonal psychotherapist (IPT) do with the evidence that suggests a cognitive behavioral model is the best treatment for a client? Ethics says I learn enough CBT to be helpful. However, I found an article from 1996 where Barber and Muenz concluded that, in some cases, IPT is similarly effective with OCD as CBT. This being the case, I would likely find interventions from CBT that focused on exposure and response prevention while continuing to take a client-centered, interpersonal approach overall.

An Adult with Moderate Depression

A second clarify case for evidence-based treatment would be an adult with moderate depression. Most of my research on this case brought me again back to cognitive behavioral therapies. In search of an equally effective model that fits with my philosophy of counseling, I found that Paradise and Kirby (2011) encourage the study of Seligman's book *Authentic Happiness* (2002) as an effective and alternate therapeutic approach to CBT. Paradise and Kirby appreciated the positive focus of Hollon's work. The authors' research hypothesis that psychotherapeutic treatments are as effective as psychopharmaceutical treatments led them to this work of Hollon and the conclusion that CBT is not the only evidence-based treatment for

depression. Similarly, and connected to Paradise and Kirby, Hollon et al. recommend from their 2002 qualitative research project, interpersonal psychotherapy (IPT) and CBT are effective alone and with psychopharmaceuticals. As such I would stick with my preferred IPT model and recommend medication management. This is a case where CBT seems more divisive and shaming than helpful. Again, I am biased toward the relational model, but have evidence that it is similarly effective as a cognitive model both from the outset and as its effectiveness is evaluated over the course of counseling.

Evaluating Counseling Effectiveness

This is the area I am most reluctant to see myself as an expert. If my treatment plan is ineffective, I tend to panic and go straight to referring clients to the experts. It was helpful for me to read a meta-analysis of 669 studies asking the question of why clients discontinue therapy (Swift & Greenburg, 2012). This study concluded that the type of therapy was not a significant factor in the early discontinuation of counseling. Client diagnosis was the most significant factor, indicating to me that I need to choose wisely the type of clients I agree to see. For example, if a couple is highly escalated, I have discovered that I am not helpful. However, if a couple is searching for better conflict management tools, I'm a Rock Star. This Swift and Greenburg study also concluded that since more experienced therapists had lower dropout rates, therapists must become more skilled at maintaining the client relationship as they progress. This says to me that I need to focus on the relational aspects of the client relationship and assess with immediacy and provider self-report.

To assess with a provider self-report, I would like to learn more about the Therapist Attunement Scale discussed in the 2020 article on how to be a secure base when relating attachment styles to attunement presentations in therapy. I understand that this scale would fit

with my relational model, my theory of change, and my desire to be effective. Continuing to reassess with this or a similar scale would allow me to hand off my secure base to clients as I prepare them for termination. Ensuring, as much as possible, that clients learn to see other secure bases (including family, friends, spouses, and God) and ask for help when needed is my current and projected method of helping them stay the course.

ACA Ethics Codes (ACA, 2014) and ACA Counselor Competencies

Ethics

As I read through the American Counseling Association code of ethics, my setting and clientele stand out to me the most. My setting (as a staff counselor in a local church) provides many opportunities for dual and overlapping relationships. I have already been faced with many of the scenarios addressed in the code. In fact, I have an entire small group of men whom I see as clients individually. Maintaining confidentiality and privacy within these relationships has proven to be a challenge. As such, Code A.6.a Previous Relationships always stands out to me. In many cases it would be impossible for me to have a clientele if I did not counsel people who sat next to me at church, had been in a small group with me, or had been in a class with me. When I read this code, and the accompanying codes, in the past I took away a dogmatic prohibition of these kinds of relationships. Many anxious moments have been spent trying to justify my clientele to my license supervisor. Upon this most recent reading, I find that I simply need to remember that it does not prohibit but gives caution to these kinds of relationships. I still need to ask what is most helpful to the client. If seeing a client who was in a class with me is the most helpful therapy for them, I need to allow myself that freedom. If taking a client who was in youth group with my children would be more harmful because of what I know of past behavior, I should refer.

Similar to the following ethical principle, Code A8 Multiple Clients warns against conflicting roles with clients who have a relationship with each other. This also can become a dilemma for me in my setting. My dilemma is mostly around couples. I sometimes have clients who recommend their spouses see me in addition to them. In this situation I typically recommend we find a separate counselor for the spouse unless we are going to move to couples counseling. Other dilemmas are easier to navigate. I need to remind myself that I can find ways to communicate confidentiality and privacy across clients. I faced this recently and I was familiarly responded to with, “we are an open book.” It would be easy to allow myself to become lax in this ethical principle with those kinds of responses, but I cannot.

Most of section C is a concern to me in staying up to date with evidence-based practices and assessment through the duration of client care. As I mentioned above, I do not feel like an expert in this area. However, I have begun a Ph.D. journey in order to more fully comprehend and adhere to this principle.

E.5 stands out as conflicting with my theoretical perspective of client-centered, interpersonal psychotherapy. This was also discussed above in relation to a question on diagnostic practices. To elaborate on this dilemma is to give a glimpse into my interaction with the other staff members I work with. Diagnosing a person with a mental illness can be seen in my circles as an excuse for bad behavior. And yet, I do not want to hide my belief nor practices from anyone. My answer to this dilemma in the past has been to leave the diagnosing to the experts. For example, if a client wanted a diagnosis for their own peace of mind, I would refer them to a local psychiatrist and let them make the diagnosis. This would allow me to remain true to my staff relationships and continue to give ethical care. I do not believe I can keep this practice up in the future. If I am going to teach and mentor diagnostics, I need to practice

diagnostics. My dilemma then becomes explaining to the vocal minority that the two disciplines of religion and science being mutually exclusive is a subjective perspective to which I do not subscribe. I see tough conversations in my future.

Competencies

Reading the *ALGBTIC competencies for counseling LGBTQIQA* is triggering. I live in a white, Anglo-Saxon, male-centric, conservative Christian culture. The question this competency brings up for me is, can a follower of Christ empathize with a member of the LGBTQIQA community? My short answer is yes; my experience proves this out. To have other members of my cultural setting hear this language is what is triggering. I agree that the LGBTQIQA and Transgender competencies are important from a human compassion standpoint. No one should be refused therapy by a counselor of their choosing. The dilemma for me is that I have to navigate the language of “positive and affirming” in a manner that does not say absorbing or integrating. I recently read an article that claimed integrating the identity of a religious group and particular sexuality (specifically gay men) was seen to decrease the amount of gay shame (Anderson & Koc, 2020). This type of language would be divisive in my world. I do feel pressure to help people in my region find a way to be helpful and not biased. I would hope we could choose to be loving rather than always choosing to be right.

One would think that a counselor in a church setting would not need to be aware of other worldviews and spiritual/religious beliefs, however, it is quite common. I find the *Competencies for Addressing Spiritual and Religious Issues in Counseling* helpful and affirming of my ability to see a variety of clients and use my own biblical worldview when a client has expressed interest in such. I do not have to quote scripture every five minutes to allow people to see Christ in me. I do not have to wear out my welcome when addressing spirituality just because I believe

all humans are spiritual creatures. I can find ways to allow clients to express their beliefs, learn from them, and challenge their attitudes and behaviors all at the same time (well, maybe not simultaneously, but certainly within the same session).

Conclusions

As I research for this paper, conceptualize its content, and read for this course, I am faced with the hard truth that I am not a fully formed counselor, educator, mentor, nor human. While this is hard truth, it is why I am determined to be a lifelong learner. I appreciate the accountability writing this paper has been for me. Accountability to refuse to ignore ethical and evidence-based practice. Accountability to be honest about where I am in my praxis and theory. Accountability in becoming the kind of counselor and educator who can be trusted and leaves a trail of healing relationships and not trauma.

References

- American Counseling Association. (2014). *ACA code of ethics*.
<https://www.counseling.org/docs/default-source/default-document-library/2014-code-of-ethics-finaladdress.pdf>
- American Counseling Association. (2009). *ALGBTIC competencies for counseling LGBTQIQA*.
https://www.counseling.org/docs/default-source/competencies/algbtic-competencies-for-counseling-lgbqiqa.pdf?sfvrsn=1c9c89e_18
- American Counseling Association. (2009). *ALGBTIC competencies for counseling transgender clients*. https://www.counseling.org/docs/default-source/competencies/algbtic_competencies.pdf?sfvrsn=d8d3732f_16
- American Counseling Association. (2009). *Competencies for addressing spiritual and religious issues in counseling*. https://www.counseling.org/docs/default-source/competencies/competencies-for-addressing-spiritual-and-religious-issues-in-counseling.pdf?sfvrsn=aad7c2c_12
- Anderson, J. R., Koc, Y. (2020). Identity integration as a protective factor against guilt and shame for religious gay men. *Journal of Sex Research*, 57(8), 1059-1068.
<https://doi.org/10.1080/00224499.2020.1767026>
- Antony, M. M., & Barlow, D. H. (2020). *Handbook of Assessment and Treatment Planning for Psychological Disorders* (3rd ed.) The Guilford Press.
- Barber, & Muenz, L. R. (1996). The role of avoidance and obsessiveness in matching patients to cognitive and interpersonal psychotherapy: Empirical findings from the Treatment for Depression Collaborative Research Program. *Journal of Consulting and Clinical Psychology*, 64(5), 951–958. <https://doi.org/10.1037/0022-006X.64.5.951>

- Beck, A. T., Steer, R. A., & Carbin, M. G. (1988). Psychometric properties of the Beck Depression Inventory: Twenty-five years of evaluation. *Clinical Psychology Review*, 8(1), 77–100. [https://doi.org/10.1016/0272-7358\(88\)90050-5](https://doi.org/10.1016/0272-7358(88)90050-5)
- Dailey, S., Gill, C., Karl, S., & Minton, C. (2014). *DSM-5 learning companion for counselors*. American Counseling Association.
- Geller, D. A., & March, J. (2012). Practice parameter for the assessment and treatment of children and adolescents with obsessive-compulsive disorder. *Journal of the American Academy of Child & Adolescent Psychiatry*, 51(1), 98-113. <https://doi-org.ezproxy.liberty.edu/10.1016/j.jaac.2011.09.019>
- Hollon, S. D., Thase, M. E., & Markowitz, J. C. (2002). Treatment and prevention of depression. *Psychological Science in the Public Interest*, 3(2), 39-77. <https://doi.org/10.1111/1529-1006.00008>
- Miller, S. D., Duncan, B. L., Brown, J., Sparks, J. A., & Claud, D. A. (2003). The outcome rating scale: A preliminary study of the reliability, validity, and feasibility of a brief visual analog measure. *Journal of brief Therapy*, 2(2), 91-100.
- Nakatani, E., Krebs, G., Micali, N., Turner, C., Heyman, I., & Mataix-Cols, D. (2011). Children with very early onset obsessive-compulsive disorder: Clinical features and treatment outcome. *Journal of Child Psychology and Psychiatry*, 52(12), 1261-1268. <https://doi.org/10.1111/j.1469-7610.2011.02434.x>
- Paradise, L. V., & Kirby, P. C. (2011). The treatment and prevention of depression: Implication for counseling and counselor training. *Journal of Counseling and Development*, 83(1), 116-119. <https://doi.org/10.1002/j.1556-6678.2005.tb00586.x>
- Prieto, L. R., & Scheel, K. R. (2002). Using Case Documentation to Strengthen Counselor

- Trainees' Case Conceptualization Skills. *Journal of Counseling and Development*, 80(1), 11-21. <https://doi.org/10.1002/j.1556-6678.2002.tb00161.x>
- Ravitz, P., & Watson, P. (2014). Interpersonal psychotherapy: Healing with a relational focus. *Focus*, 12(3), 275-284.
- Rowe A. C., Gold, E. R., & Carnelley, K. B. (2020). The effectiveness of attachment security priming in improving positive affect and reducing negative affect: A systematic review. *International Journal of Environmental Research and Public Health*, 17(3). <https://doi.org/10.3390/ijerph17030968>
- Schore, J. R., & Schore, A. N. (2008). Modern attachment theory: The central role of affect regulation in development and treatment. *Clinical Social Work Journal*. 36(1), 9-20. <https://doi.org/10.1007/s10615-007-0111-7>
- Swift, J. K., & Greenberg, R. P. (2012). Premature discontinuation in adult psychotherapy: A meta-analysis. *Journal of Consulting and Clinical Psychology*, 80(4), 547-559. <https://doi.org/10.1037/a0028226>
- Talia, A., Muzi, L., Lingiardi, V., & Taubner, S. (2020). How to be a secure base: Therapists' attachment representations and their link to attunement in psychotherapy. *Attachment and Human Development*, 22(2), 189-206. <https://doi.org/10.1080/14616734.2018.1534247>
- Yourceus.com. (2022). *Cfn9885 - section 2: components of effective counseling*. <https://www.yourceus.com/pages/cfn8685-section-ii-components-of-effective-counseling#:~:text=The%20basic%20stages%20of%20counseling,termination%2C%20and%20follow%20Dup>

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What a delightful paper to read Raymond. You've been honest, thoughtful, and sincere. I am excited for you and your doctoral adventure. I believe you are going to flourish, especially as you manage the complexity of what we do as Christians in a fallen and now predominantly secular society. I am confident you will excel if you continue to apply yourself in the manner you did in this, your first paper in the program.